

Still Here: A Guide to Waiting

A Practical Guide to Surviving the Gap Between Test and Result



Introduction: What This Guide Is Actually For

The waiting isn't the hard part. The hard part is the hours that follow—sitting in your kitchen, staring at your phone, watching your mind turn a three-day window into a prison of rehearsed conversations, constructed catastrophes, and imaginary appointments with doctors who say words you haven't heard yet.

You know it's a waste. You know you're losing days. You know, on some level, that the rumination isn't protecting you—it just feels like trying.

Here's what nobody taught you: you can be deeply uncomfortable and still function. You can hold "this is out of my hands" and not collapse under it. You can survive the wait without your mind eating itself alive.

This guide teaches you the specific psychological skills that make the gap between test and result survivable—not by convincing you to stop worrying, but by showing you what to do with the worry once it's there. You will not think your way out of this. You will act your way through it. And it turns out there are real, learnable actions for that.

Still Here is organized into four parts:

Part One gives you tools for the acute moments—the times when anxiety pulls you into a spiral and you need an interruption right now, in your body, in under thirty seconds.

Part Two addresses the deeper work: accepting that this situation is outside your control without confusing acceptance with defeat. This is where most waiting-related suffering lives, and it's also where the most meaningful change happens.

Part Three focuses on your actual life—the days that still exist while you're waiting. You'll learn how to structure unstructured time, set information boundaries, and keep enough forward momentum that the wait doesn't swallow your entire existence.

Part Four handles the human part: staying connected to people while your instinct is to pull away, communicating when you don't know what to say, and giving yourself permission to be less available without guilt.

Each chapter ends with a concrete action step—something you can try today, with specific instructions. Some of these will feel too simple to work. That's normal. The skills in this guide aren't about finding the perfect solution; they're about giving your nervous system something to hold onto when the uncertainty feels like it's going to pull you under.

You don't need to read this guide in order. If you're in the middle of a panic spiral right now, start with Chapter 3 (Body First, Mind Follows). If you've already lost a few days to rumination and need to pull yourself back, Chapter 2 (Name It to Tame It) will help. If you're reading this before a scheduled wait and want to prepare, start at the beginning and work through.

Whatever brought you here—you're still here. That's what this guide is for.



Part One: Ground Before You Spiral

The Physiological Interruption — What to Do the Moment You Feel the Pull

When anxiety spikes during a waiting period, your nervous system doesn't wait for your permission. It kicks into threat-response mode: heart rate up, breathing shallow, muscles tense, thinking narrows to the danger. Your brain's amygdala—responsible for detecting threats—has essentially hijacked the rational prefrontal cortex, and now you're not problem-solving anymore. You're rehearsing worst-case scenarios.

This isn't a character flaw. It's a feature of human neurology that worked fine when the threats were tigers. For a three-day wait between a blood draw and results, it's considerably less useful.

The problem is that by the time you notice you're spiraling, you're usually already deep in it. The catastrophic thought has been running for ten or twenty minutes before you catch yourself. You've mentally rehearsed the doctor's call, constructed three alternative conversations, and started googling survival statistics for conditions you don't have.

Part One gives you tools to interrupt that pattern earlier—before the spiral gains momentum. These aren't relaxation techniques. They're more like pulling a fire alarm: not a solution, but a disruption. A chance to reset before the fire spreads.

Chapter 1: The Six-Second Anchor

The Breath-Counting Technique You Can Deploy Anywhere

Here's the situation: you're sitting at your desk. A work meeting just ended. Your phone is in your hand, and you're staring at the patient portal login page for the third time in an hour, even though you know nothing has changed. Your jaw is tight. Your shoulders are somewhere near your ears. You're already mentally at the appointment you don't have yet, hearing words nobody has said.

This is the moment. Not when you're already crying, not when you've been spiraling for two hours—right now, with the pull just starting to feel like something you should probably interrupt.

The six-second anchor is a breath-counting technique designed specifically for this moment. It works because it requires your prefrontal cortex to handle a task—counting—while simultaneously slowing your exhale, which activates the parasympathetic nervous system (the "rest and digest" mode). You're not eliminating the anxiety. You're interrupting the forward momentum before it becomes a full rehearsal.

The Three-Step Sequence

Step 1: Close your mouth and inhale through your nose for a count of four. Don't try to breathe deeply. Just a normal inhale, extended to a count of four. If four feels impossible, start with three. The point isn't the number; it's the directed attention.

Step 2: Hold your breath for a count of four. This part matters more than most breathing exercises admit. The hold allows the carbon dioxide to build slightly, which makes the next exhale feel more release-like. If four feels like too long, go to two. Consistency with the technique matters more than matching an ideal count.

Step 3: Exhale slowly through your mouth for a count of six.

Make the exhale longer than the inhale. This is the part that activates the parasympathetic system. Imagine you're fogging a mirror—slow, steady, deliberate.

Repeat this three times. That's eighteen seconds. By the third cycle, your heart rate will have started to shift. The spiral momentum will have broken. You'll be back in your body, sitting at your desk, with the patient portal still there but no longer pulling you in.

Why This Works (And Why You Don't Believe It Will)

Your nervous system can't maintain a full threat response while you're performing a cognitive task like counting. It's like trying to panic and calculate a tip simultaneously—the systems compete for bandwidth. By giving your prefrontal cortex something to do, you pull resources away from the amygdala's catastrophic loop.

The caveat: this won't make the anxiety go away. It won't solve the underlying problem. What it does is create a brief window—maybe sixty seconds, maybe five minutes—where you're not actively making things worse. That's the goal. You're not trying to feel good. You're trying to stop the compounding.

Your Action Step for Today

The next time you notice yourself pulled toward the phone, the portal, the google search—before you give in—do three cycles of 4-4-6 breathing. Set a timer on your phone right now to remind you to practice this once an hour. You're not building a habit yet; you're just getting one exposure to what it feels like to interrupt the pattern before it starts.

Chapter 2: Name It to Tame It

The Structured Labeling Practice That Breaks the Rumination Loop

Here's what happens in a rumination spiral: your worried thoughts have content. They're about something specific—a test result, a diagnosis, a future you're trying to prepare for. And that content feels so important. If you just think through it carefully enough, you'll be ready. You'll have a plan. You won't be caught off guard.

Except you won't. You'll never think your way to readiness on a topic where the information doesn't exist yet. And the act of "thinking through" is actually your brain mistaking rehearsal for problem-solving. The emotional engagement with catastrophic content—feeling the fear, rehearsing the conversation—gets interpreted by your nervous system as productive action. Which makes you do more of it.

Labeling interrupts this by changing the object of your attention. Instead of following the content of your worried thoughts, you practice identifying the **type** of thinking happening and naming it. This is a technique adapted from mindfulness-based cognitive therapy and Acceptance and Commitment Therapy, and the research on it is surprisingly robust: naming an emotion or cognitive pattern reduces its intensity by giving the prefrontal cortex something analytical to do.

The Five Thought Categories

When you notice yourself pulled into rumination, pause and ask: what kind of thinking is this? Use the reference below:

CATEGORY	WHAT IT LOOKS LIKE	THE LABEL TO USE
Prediction	"The result will be bad. I can feel it."	"I'm predicting the future again."
Catastrophizing	"If this is what I think it is, everything falls apart."	"I'm catastrophizing."
Fortune-telling	"The call will come at 4pm, and I'll be alone."	"I'm fortune-telling."
Mind-reading	"The doctor will think I'm dramatic. They won't take me seriously."	"I'm mind-reading."
Reassurance-seeking	"Let me check the forums one more time. Someone will know something."	"I'm reassurance-seeking."

How to Use the Labels

When you catch yourself spiraling, say the label out loud if you can. If you're in public or don't want to look like you're talking to yourself, whisper it. The physical act of verbalizing the category matters—it's harder for your brain to stay in emotional mode while producing language.

After you name it, you don't have to stop the thought. You just have to recognize that you're in a **type** of thinking, not a factual inquiry. The thought isn't information. It's a pattern.

Here's an example: You're lying awake at 2am, running through your family's medical history, looking for clues. Instead of following the content ("maybe it's genetic, maybe it skips a generation, maybe my father had this and just didn't know—"), you pause and say to yourself: "I'm reassurance-seeking. I'm trying to find information that will make me feel better, but the information doesn't exist yet."

The thought doesn't disappear. But the pull toward continuing the search loses some of its force.

Your Action Step for Today

Print or write out the five-category reference card. Keep it somewhere you'll see it—a bathroom mirror, a desk drawer, a phone note you scroll to frequently. The first few times you use it, you'll probably identify the category after you've already spent twenty minutes spiraling. That's fine. The goal for today is just to practice recognizing the categories at all. Tomorrow, try to catch yourself a little earlier. The skill builds.



Chapter 3: Body First, Mind Follows

The 90-Second Body Scan Protocol for Acute Anxiety

There is a specific reason rumination feels safer than paying attention to your body: your body is actually doing something in anxiety. Your heart is beating faster. Your stomach might be nauseous. There might be a tightness in your chest that you're scared to look at directly. The mental rehearsal feels contained; the body feels like it might be signaling something catastrophic.

This is why rumination can persist for hours—you're choosing the familiar discomfort of your worried thoughts over the unfamiliar discomfort of what's happening in your body. But the body isn't lying to you. The physical sensations of anxiety are uncomfortable, not dangerous. And the only way out is through.

The **90-second body scan** is a protocol designed to interrupt the anxiety loop by forcing your nervous system to register the present moment. It's not relaxation. It's not a spa experience. It's a

deliberate redirection of attention from the mental rehearsal to the actual physical reality of your body right now, which exists in this room, at this moment, regardless of what the test results might say.

The Step-by-Step Body Grounding Script

Find a position where your feet are flat on the floor. Sit in a chair or on the edge of your bed. You can do this with your eyes open or closed—closed is more effective for most people, but open is fine if you feel unsafe with your eyes closed.

Phase 1: Feet and Legs (30 seconds) Notice the weight of your legs pressing into the chair or bed. Feel the floor under your feet. Is it cold? Warm? Are your toes curled or relaxed? You don't need to change anything—just notice. Name what you feel: "pressure in my thighs, cool floor under my heels."

Phase 2: Torso and Breathing (30 seconds) Bring attention to your stomach area. Don't try to breathe differently. Just notice your stomach rising and falling. Is it shallow or deep? Fast or slow? Move attention slowly up to your chest. Feel your ribs expand. Place a hand on your chest if it helps you focus. Notice your heartbeat without judging it.

Phase 3: Head and Hands (30 seconds) Notice your jaw. Is it clenched? Relaxed? Let your tongue rest in the bottom of your mouth. Notice your hands—are they gripping something? Rest them open on your thighs. Feel the temperature of the air on your face. Notice one thing you can hear that's not your thoughts.

Why This Works

The body scan works because anxiety is partly a failure of interoception—the ability to accurately sense your body's internal states. During prolonged stress, people often become either hyper-aware of body sensations (interpreting every twitch as a symptom) or disconnected from them entirely (dissociation as a coping mechanism). The body scan trains interoception in a controlled way. You learn that physical sensations are just sensations—pressure, warmth, tingling—without catastrophic meaning.

Additionally, sustained attention to body sensations activates the insula and anterior cingulate cortex—parts of the brain associated with self-awareness and emotional regulation. You're essentially doing a targeted workout for the parts of your brain that anxiety weakens.

Your Action Step for Today

Do the full 90-second body scan once today, preferably when you're not already spiraling. Read through the script once, then perform it without the script—you'll remember the phases. Set a daily reminder at a consistent time (morning works well) to practice this. You're not doing it to feel calm; you're doing it to build the skill of redirecting attention from mental content to physical reality. After a week of daily practice, you'll notice it works faster.



Key Takeaway for Part One: The spiraling doesn't have to become a full collapse. You have physiological tools—breathing, labeling, body scanning—that interrupt the pattern before it compounds. Practice these when you're not in crisis so they're available when you are.



Part Two: Acceptance Is a Skill, Not a Surrender

Shifting Your Relationship with the Uncontrollable

Here's where most waiting-related suffering actually lives.

You've done the breathing exercises. You've named your thought patterns. You've interrupted the spiral a few times. But underneath all that technique, something is still running: a low-grade, background-level conviction that if you just think hard enough,

prepare well enough, worry thoroughly enough, you can affect the outcome. That the waiting is something you should be doing something about.

This is the belief that drives most of the lost days. And it's not entirely wrong—humans are meaning-making creatures. We want to believe our actions matter. But there's a difference between "I want to be prepared for all outcomes" and "I can control an inherently uncontrollable situation." One is adaptive. One is a recipe for suffering.

Part Two is about the harder work: learning to genuinely accept that this situation is outside your control—not because you don't care, but because the alternative is burning energy on actions that produce nothing but exhaustion.

Acceptance is not resignation. It's not giving up. It's the difference between pushing against a wall with all your strength (exhausting, pointless) and stepping to the side of it (still aware the wall exists, but no longer fighting it).



Chapter 4: The Uncontrollable Inventory

The Sorting Exercise That Prevents Rehearsal of Impossible Actions

The mind, during a waiting period, is remarkably creative about generating actions you could take. You could call the doctor's office and ask if they have preliminary results. You could search for the exact diagnostic criteria for your suspected condition. You could draft a list of questions for the appointment. You could research second opinions, dietary changes, holistic interventions.

Some of these are reasonable. Most are attempts to feel like you're doing something because doing something feels like you're affecting the outcome. But many of them—checking the portal obsessively, researching the condition, drafting catastrophic

contingencies—are rehearsals for actions that either can't be taken yet (the test isn't processed) or don't need to be taken (the result might be fine).

The **Uncontrollable Inventory** is a concrete sorting exercise. Its purpose is to externalize the "what's in my control" question so your mind isn't running it on loop internally. By writing it down, you make the distinction between control and no-control concrete, visual, and something you can return to.

The Two-Column Worksheet

Take a sheet of paper or open a document. Draw a vertical line down the middle. On the left, write **IN MY CONTROL**. On the right, write **NOT IN MY CONTROL**.

Now, for your specific situation, fill in the categories. Pre-loaded prompts for a medical waiting scenario:

IN MY CONTROL

- How I spend the hours of this day
- Whether I check the portal (and how often)
- What I eat, whether I exercise
- Who I talk to and what I tell them
- How much news/information I consume about my suspected condition
- My breathing, my posture, my body
- Whether I practice the skills in this guide
- Showing up to the appointment (if one is scheduled)
- Asking clear questions when I have information to ask about

NOT IN MY CONTROL

- The test results themselves
- When the results arrive
- What the results say
- The doctor's assessment and recommendations
- What my family history predicts
- What happened to other people with similar symptoms
- Whether the lab has processed my sample

- What the specialist decides to do next
- Other people's reactions to my situation

How to Use the Inventory

Once you've completed it, read through the "Not In My Control" column out loud. Read through it again. Then notice how you feel. Probably uncomfortable—the right-hand column contains the things your mind keeps trying to rehearse and prepare for, and seeing them listed makes the reality of the situation more vivid.

But notice also what's in the left column. There's actually quite a lot there. This is your actual domain of action. This is where your energy produces results.

The Inventory isn't a pep talk. It's a sorting mechanism. When you notice your mind generating a catastrophic action plan at 11pm, you can check the right-hand column: "Is this something I can actually do right now? Or am I rehearsing an action that belongs in the 'not in my control' category?" The answer will often be obvious.

Your Action Step for Today

Complete the Uncontrollable Inventory right now, specific to your current situation. Don't rush it—this is cognitive work, and going slowly makes it stick. If you find yourself filling the left column with vague things like "stay positive" or "not stress," push harder for specifics: "I can decide not to google symptoms between now and dinner. I can choose to call one friend today. I can do three cycles of breathing before I open the patient portal." Specific actions are what you're after.

Chapter 5: Dropping the Rope

What Happens When You Stop Fighting the Uncertainty

Imagine you're holding one end of a rope. The other end is attached to a car that's trying to pull away. You're planted, resisting, pulling back with everything you've got. The car keeps going. You keep pulling. Eventually you're dragged forward, exhausted, probably on the ground.

This is what fighting uncertainty looks like. The uncertainty isn't going to give up. The car isn't going to stop. You're not going to "win" by pulling harder. And the energy you spend on the resistance is energy that doesn't go anywhere useful.

Dropping the rope is a metaphor from relational frame theory (the psychological framework underlying Acceptance and Commitment Therapy). The idea is simple: at some point, the most useful action is to let go. Not because you've lost. Not because you don't care about the outcome. But because continuing to pull produces nothing except your own exhaustion.

The Difference Between Acceptance and Resignation

This is where people get stuck. "Dropping the rope" sounds like giving up, like accepting defeat, like telling yourself the results don't matter. That's resignation.

Acceptance is different. Acceptance means: I know the results matter. I'm not okay with whatever they turn out to be. I wish this weren't happening. But my wishing isn't changing the situation, and my fighting isn't either. So I am going to hold the outcome loosely, in full awareness of what it might mean, without letting the rope drag me into the ground.

Resignation says: nothing matters anyway, I give up. Acceptance says: this matters, and I can't control it, so I'll hold it with open hands.

The emotional posture is entirely different. Acceptance preserves your vitality. It keeps you available for the life that's still happening, even in the middle of the wait. It doesn't mean you're not scared. It doesn't mean you're not hoping. It means you're not exhausting yourself fighting something that cannot be defeated.

The Daily Re-Anchoring Prompt

One practical tool for "dropping the rope" is a daily re-anchoring practice. Every morning (or evening, or both), ask yourself:

"What is the one thing I can actually do today?"

Not "what can I do about the results"—those are out of your control. Not "what can I do to prepare for every possible outcome"—that's the rope-pulling. The question is narrower: what is one concrete action available to you right now, in this body, in this day?

It might be: eat breakfast. Walk around the block. Send one text to a friend. Sit with the breathing exercise. Finish a work task. Take a shower. The specificity matters. By asking what you *can* do—and naming one thing—you redirect from the impossible domain (results, outcomes, futures) to the possible domain (this hour, this action, this moment).

Your Action Step for Today

Tonight, before you go to sleep, write down the answer to "What is the one thing I can actually do tomorrow?" Keep it to one thing. Make it concrete. The next morning, do that one thing before you do anything else. This is not a productivity system; it's a daily re-commitment to the possible. The uncertainty is still there. You're just choosing not to pull the rope about it.

Chapter 6: Uncontrollable Rituals

Daily Practices That Give Anxiety Somewhere to Live

Here's a truth nobody tells you during a medical wait: the anxiety doesn't go away because you accept the situation. It's still there. The physiological arousal, the background hum of worry, the sudden spikes of fear when you see something that reminds you of your situation—these don't disappear because you've done the Uncontrollable Inventory. They're still running.

The mistake is thinking that rituals will suppress the anxiety. That's not what they're for. Rituals give the anxiety a container—a structured, intentional place where it can exist without taking over your entire day. They're not eliminating the energy; they're channeling it.

The word "ritual" here doesn't mean superstition or compulsion. It means a deliberate, sensory-rich, repeatable practice that creates a sense of agency and structure during a period when the future is unknown. The key features are intentionality (you choose to do this), sensory engagement (you're in your body, using your senses), and consistency (it happens at the same time, in the same way, which creates predictability in an unpredictable situation).

Three Pre-Written Ritual Options

Morning Ritual (5-7 minutes): The Sensory Wake-Up This ritual is designed for the early hours, when anxiety spikes before your brain is fully online. It grounds you in your body before the day pulls you into mental rehearsal.

1. Before you reach for your phone, sit on the edge of your bed with your feet flat on the floor.
2. Take three cycles of 4-4-6 breathing.
3. Wash your face with cool water. Feel the temperature change. Notice the sensation.
4. Drink a glass of water. Don't gulp it—sip it. Notice your throat, the temperature, the swallow.

5. Set one intention for the day: "Today, I will [one concrete thing you can do]."
6. Only then reach for your phone—and only for a specific purpose (a calendar check, a weather check). No portal. No forums.

Mid-Day Ritual (2-3 minutes): The Check-In This ritual interrupts the mid-afternoon slump, when rumination often spikes as energy depletes. It's designed to be done at a desk or in a bathroom, without drawing attention.

1. Pause wherever you are. Put your hands flat on the desk or your thighs.
2. Do a 30-second body scan: feet, legs, torso, hands, face. Name what you feel.
3. Ask yourself: "Am I in the present, or am I somewhere that doesn't exist yet?" If you're somewhere that doesn't exist yet (rehearsing, catastrophizing), say the label out loud: "I'm [prediction / catastrophizing / fortune-telling]."
4. Return your attention to what's in front of you: your screen, your work, the room you're in.
5. Take one normal breath. Continue.

Evening Ritual (10-15 minutes): The Wind-Down This ritual is for the hours when the anxiety intensifies and the night feels long. It's not about eliminating worry; it's about containing it to a specific window.

1. Set a time boundary (e.g., 9pm) after which you will not check the portal, google symptoms, or research your condition.
2. At or before that time, write three sentences: "Today I did [one thing]. Today I felt [one emotion, even uncomfortable ones]. Tomorrow I will [one thing I can do]." This externalizes the day and creates a record of forward movement.
3. Do one sensory-rich activity for at least ten minutes: a bath or shower, cooking a simple meal, sitting with a cup of tea (not looking at your phone), gentle stretching, coloring, playing an instrument. The activity should engage your hands and senses.

4. When you go to bed, keep a notepad nearby. If worrying thoughts arise, you can write them down—not to solve them, but to signal to your brain that they've been "captured" and you can set them aside until morning.

Your Action Step for Today

Choose one of the three rituals above. Commit to doing it for the next seven days without missing more than one day. Set a reminder on your phone if needed. Rituals work because of repetition and consistency; you're not looking for a profound experience on day one. You're building a container.



Key Takeaway for Part Two: Acceptance isn't defeat—it's the decision to stop fighting something you can't change so you have energy for what you can. The rituals in this chapter don't eliminate anxiety; they give it a place to exist without taking over your life.



Part Three: Structuring the Unstructured Time

Your Life Doesn't Pause for Waiting

Here's the thing nobody prepares you for: while you're waiting, the rest of the world doesn't stop. Work continues. Relationships continue. The calendar pages turn. Meals still need to be made. Bills still arrive. The dog still needs walking. And the hours you lose to rumination aren't coming back.

This is where the real cost of the wait becomes visible—not just the emotional suffering, but the functional loss. The days you spent staring at your phone were days you could have spent working,

connecting, resting, creating. They were infrastructure days, too: maintenance for the human systems that keep your life running. And they're gone.

Part Three is about keeping the infrastructure running. Not about ignoring the wait or pretending you're fine. About making sure that however this turns out, you haven't hollowed out the days leading up to it.



Chapter 7: The Forty-Five Minute Window

Time-Boxing the Unstructured Day

When you're in a waiting period, time develops a strange quality. Hours stretch and compress unpredictably. You look up and it's 2pm and you haven't eaten. You look up again and it's 9pm and you've accomplished nothing and the night is just beginning. The lack of external structure—because you're not going anywhere, you're not meeting anyone, you're just waiting—creates a kind of temporal paralysis.

Time-boxing is a project management technique adapted for personal use. The core principle is simple: you divide your day into defined windows, each with a specific purpose. The window is bounded (it has a start and end time) and defined (you know what you're doing in it). This creates structure from the outside, so your depleted motivation doesn't have to generate it internally.

How to Build Your Forty-Five Minute Windows

Step 1: Identify three to four non-negotiable windows for the day.

Examples: Morning work block (9am-12pm), afternoon task window (1pm-4pm), evening wind-down (7pm-9pm). These are your containers. They don't have to be filled with productive work—they can include rest, ritual, the body scan. The point is that they have boundaries.

Step 2: Within each window, use 45-minute cycles. Work or engage in a task for 45 minutes. Then take a 10-15 minute break. During the break: no portal-checking, no googling, no rumination. The break is for the body scan, a walk, a cup of tea, the breathing exercise. Then return.

Step 3: Schedule one "worry window" per day. This is counter-intuitive but essential. Pick a specific time (e.g., 6pm-6:15pm) and tell yourself: I am allowed to worry during this window. I can check the portal, I can google, I can spiral—all of it is permitted, but only during this window. Outside of it, the rule is: not now.

The worry window works because it gives the reassurance-seeking impulse a legitimate home. You don't have to suppress the urge to check; you just redirect it to a scheduled time. And because the window is bounded, it can't expand to fill the whole day.

Step 4: End the day with a shutdown ritual. Before you go to sleep, close the loops: "I did [one thing] today. The wait is still happening, but I made it through the day." This isn't toxic positivity; it's a factual accounting that counters the rumination tendency to see the day as entirely lost.

Your Action Step for Today

Map out your ideal day using the 45-minute window structure. Identify your non-negotiable windows, schedule one worry window, and plan your shutdown ritual. Tomorrow, try to follow it. If you don't follow it perfectly, note where the structure broke (not as self-criticism, but as data). Adjust tomorrow.

Chapter 8: The Information Diet

What to Consume, What to Cut, and How to Set Limits with Yourself

During a medical wait, information starts to feel like oxygen. The logic goes: if I just know enough, I'll be prepared. If I research the condition thoroughly, I'll have more control. If I read enough forums, I'll find someone who went through this and everything turned out fine.

This is reassurance-seeking wearing the disguise of preparation. And the problem isn't that information is bad—it's that during a waiting period, your brain is primed to interpret information catastrophically. You read a case study and your mind grabs the worst outcome. You read a forum thread and your mind selects the person whose symptoms matched yours. You google survival statistics and your mind does the math on you.

This is called **confirmation bias**: you remember the information that confirms your feared prediction and discard the information that doesn't. So the more you consume, the worse you feel—not because the information is making you sick, but because your anxiety is cherry-picking it.

The solution isn't to become uninformed. It's to become intentional about what you consume, when, and why.

The Information Diet Framework

Tier 1: Necessary Information (Low Volume, Scheduled) This includes: the facts your doctor has given you about the test and what it looks for, the logistics of appointments (time, location, what to bring), and any direct communication from your healthcare team. This information is useful, factual, and low in anxiety-trigger potential. Consume it during your worry window or during a specific "logistics check" time. Not before bed.

Tier 2: Contextual Information (Moderate Volume, Purposeful)

This includes: basic educational material about the condition being tested for, written by reputable sources (not forums). If you need to understand what the test measures and why it was ordered, one or two high-quality sources are sufficient. Read them during a specific time (not at night) with the body scan available as a resource if anxiety spikes.

Tier 3: Exploratory Information (None During the Wait) This includes: forums, Reddit threads, Google searches triggered by specific symptoms, reading case studies, comparing your situation to other people's situations, researching alternative treatments before you have a diagnosis. This information is almost universally harmful during a waiting period. It doesn't prepare you; it feeds your rumination. Delete the apps if you need to. Block the websites. Tell a friend to change your password so you can't log in without accountability.

The 24-Hour Rule: Before you consume any information about your condition outside your scheduled windows, ask: "Am I doing this because I need this information right now, or because I'm looking for relief from anxiety?" If it's the latter, don't do it. Wait 24 hours. If you still want to know it after 24 hours, schedule it for your worry window.

Your Action Step for Today

Audit your current information consumption: what apps are on your phone, what websites you visit, what forums you've been reading. Identify which tier each one falls into. For Tier 3 sources, take one concrete action today: delete the app, use a website blocker, or ask a trusted person to change your passwords. You don't need to be dramatic about it—just make the exploratory searching slightly harder to do on impulse.

Chapter 9: The Minimum Viable Day

Doing Enough Without Falling Apart

There's a concept in startup culture called the "minimum viable product"—the smallest version of something that actually works. You can apply this to days during a waiting period.

The minimum viable day is the smallest version of a functioning day you can manage while waiting. It's not a day where you crush your to-do list, meditate for an hour, exercise, eat perfectly, and be productive at work. It's a day where you: eat something, move your body in some way, do one work task or one household task, and go to bed without having spent the entire day spiraling.

This isn't settling. It's calibration. During a waiting period, your capacity is legitimately reduced. You're running a background process that consumes cognitive and emotional resources. Trying to maintain your normal output level is a recipe for collapse. The goal is to maintain enough function that the infrastructure holds: you eat, you sleep (some), you don't lose your job, you don't isolate completely.

The Minimum Viable Day Checklist

You can print this out and check it daily:

Sustenance: Did I eat at least one real meal today?

Movement: Did I move my body in some way today? (A walk, stretching, walking to the mailbox—anything counts.)

One task: Did I complete one work task, household task, or meaningful activity today? (It doesn't have to be productive by normal standards. It just has to be done.)

Connection: Did I have at least one human interaction today? (A text, a phone call, seeing someone—even a short one counts.)

Ritual: Did I do at least one of the rituals from Chapter 6 today?

Wind-down: Did I do a body scan or grounding exercise before bed?

If you hit four out of six on a given day, that's a successful day. The goal isn't perfection; it's enough.

Your Action Step for Today

Print or copy the Minimum Viable Day checklist. Put it somewhere visible—on your refrigerator, next to your bed, as a note on your phone. For the next seven days, check in at the end of each day. Don't judge yourself; just track. If you notice a pattern (you're consistently missing movement, or you're isolating more than usual), that's information, not failure.



Key Takeaway for Part Three: The wait doesn't have to hollow out your days. Structured time, an information diet, and a realistic minimum bar let you maintain your life—your work, your relationships, your health—without pretending the wait isn't happening.



Part Four: Staying Connected While Pulling Away

The Human Part of Waiting Alone

Here's the part that nobody talks about: while you're waiting, you probably want to be alone. Not because you don't love the people around you, but because the anxiety is exhausting and you don't want to explain it. Because you're afraid people will get tired of hearing about it. Because you don't know what to say, and silence feels safer than fumbling through a conversation about something you can't control.

This is completely normal. It's also dangerous.

Isolation during a waiting period doesn't just feel lonely—it actively makes the rumination worse. The anxious mind, left to its own devices, compounds and spirals. Other people's presence—even without conversation—provides a reality check. Someone else's normal energy is a gravitational pull back toward the present. And the conversations you don't have because you've pulled away become the thing you wish you'd done, regardless of how the wait ends.

Part Four is about staying connected without burning yourself out. It's about communicating when you don't know what to say, accepting help you don't know how to ask for, and giving yourself permission to be less available without guilt.



Chapter 10: The Support Calibration

Matching Your Social Needs to Your Social Capacity

During a waiting period, your social capacity fluctuates wildly. Some days you want to talk about it; some days the subject feels unbearable. Some days you want company without conversation; some days you want to be completely alone. And the people around you have their own schedules, their own anxieties, their own capacity.

The mistake most people make is binary: either they're talking to everyone about everything, or they're completely silent. Neither is sustainable.

The Support Calibration is a framework for matching your social needs to your social capacity on any given day. It's not a fixed plan; it's a daily check-in.

How to Calibrate

Every morning, ask yourself:

1. How much do I want to talk about what's happening? (Scale of 1-10)
2. How much social interaction can I tolerate today? (Scale of 1-10)
3. What kind of interaction would be most useful? (Vent / Distraction / Presence / Practical help)

Then, based on your answers, reach out accordingly:

- If you want to talk but can only tolerate low interaction: send a voice memo or text to one person. Don't call. Give them a heads-up: "I need to vent, is now a good time?" This lets them prepare.
- If you want distraction but can tolerate interaction: suggest a specific activity (a walk, a coffee, watching a show together). The activity gives you something to do besides talk.
- If you want presence without conversation: ask someone if you can just be in the same room while they do their own thing. You don't have to talk. You don't owe them an explanation.
- If you don't want any interaction: tell one person that you're stepping back for the day. You don't have to explain why. "I'm taking a social break today" is sufficient.

The one-person rule: Identify one person—partner, close friend, family member—who you give permission to check in on you. This person is allowed to text you every day, even if you don't respond.

They don't need a response. They just need to know you're alive. This prevents complete isolation without requiring you to manage multiple relationships.

Your Action Step for Today

Identify your one person. Text them: "I'm going through something hard right now, and I might not be very responsive. Can I ask you to check in on me—maybe one text a day—even if I don't reply?" Most people will be relieved to have something concrete to do. You are giving them the gift of action.



Chapter 11: Communication Without Disclosure

How to Stay Present with People When You Can't Tell Them Why

The hardest part of waiting is that you often can't tell people why you're distracted, short-tempered, or absent. Maybe you don't have a diagnosis yet. Maybe you don't want to worry people before you know anything. Maybe the situation is private. So you show up to dinner, to work, to the school pickup, and you have to be a person when part of you is screaming.

This is exhausting. And it leads to one of two responses: over-disclosure (telling people more than you wanted to, because the pressure to explain builds) or complete withdrawal (disappearing from social life entirely).

Communication without disclosure is a middle path. It's the skill of being present with people—genuinely present, not just physically present—while holding your private situation as your own. It doesn't require lying. It requires boundaries.

The Three Boundaries Framework

Boundary 1: You don't owe a performance. If someone asks "how are you?" and you're not okay, you don't have to say "fine" in a bright voice. You can say "honestly, I'm having a hard day—I'm dealing with something and I'm not up for a lot of conversation, but it's good to see you." This is honest without being disclosure-heavy. It signals that you're not fine without requiring explanation.

Boundary 2: You can redirect without explaining. If the conversation is pulling you somewhere you don't want to go—questions about your health, your future, your plans—you can redirect without explanation: "I'd rather not talk about that right now—can we focus on [something else]?" or "I'm not ready to discuss that, but I appreciate you asking." Most people will respect this. Those who push deserve a firmer boundary.

Boundary 3: Presence is enough. You don't have to be engaged, animated, or contributing meaningfully to be a good friend, partner, or colleague. Sometimes your presence is the only thing you can offer, and it's enough. Sitting next to someone, watching a movie, being in the room while someone else talks—these are all forms of connection that don't require you to perform wellness you don't feel.

Your Action Step for Today

The next time you're in a social situation during this wait, try one boundary from above. If someone asks how you are and you're not okay, practice saying "I'm having a hard day, but I'm here." Notice how it feels. Notice whether the sky falls. Most of the time, it doesn't.

Chapter 12: Permission to Be Unavailable

Guilt-Free Boundaries and the People Who Matter

Here's the guilt trap: you know you should stay connected. You know isolation is bad for you. You know your friends and family care about you. But every time you decline an invitation or cut a conversation short, the guilt arrives: You're being a bad friend. They're trying to help. You're lucky to have people who care. You should be grateful.

The guilt is a liar.

The guilt assumes that your primary obligation is to other people's needs—even when your own capacity is genuinely reduced. During a medical wait, you have a temporary but legitimate reduction in your social bandwidth. Declining invitations, limiting conversations, and pulling back from social energy isn't selfishness; it's triage. You're protecting enough energy to get through the day.

The challenge is that guilt doesn't feel like triage. It feels like failure. So you push yourself to attend events you don't want to attend, to answer calls you don't have the energy for, to pretend you're fine when you're not. And at the end of the day, you're exhausted, and the wait is still there, and you've spent your limited resources on maintaining appearances instead of maintaining yourself.

Permission to be unavailable is not giving up on people. It's recognizing that the people who matter will understand—and that the people who don't understand weren't going to be there for you anyway.

How to Grant Yourself Permission

Reframe the math: When you decline something, don't frame it as "I'm letting them down." Frame it as "I'm protecting my capacity for the people I can show up for." A short, limited presence with

someone else is often more genuine than a depleted, resentful full presence. Quality over quantity—applied to relationships, not just products.

Communicate proactively: You don't have to explain your situation to everyone. But if you're close enough that they'll notice your absence, give them a heads-up: "I'm going through a hard time and I'm going to be less available for the next few weeks. I still care about you; I just need some space." Most people will honor this. Those who pressure you after a clear boundary are telling you something about themselves.

Practice the phrase: "I need to take care of myself right now. Can I get back to you / take a rain check / just have some quiet time?" This is a complete sentence. You don't need to add details. You don't need to apologize extensively. "I need this" is sufficient.

Distinguish between guilt and grief: Sometimes the bad feeling you have when you decline isn't guilt—it's grief. You're grieving the loss of normalcy, the ability to be a present friend, the version of yourself that wasn't carrying this weight. Grief deserves acknowledgment, not dismissal. If you can name it, you can hold it without letting it push you into over-commitment.

Your Action Step for Today

Identify one social obligation you have coming up that you don't want to fulfill. Instead of pushing through or canceling entirely, try this: reach out and offer a modification. "I can't do dinner, but could we do a quick coffee instead?" or "I need to skip this one, but can I call you this weekend?" This is a middle path. You're staying connected, but on terms that work for you.



Key Takeaway for Part Four: You don't have to disappear to protect yourself, and you don't have to perform wellness you don't feel. A few intentional connections, clear boundaries, and permission to pull back—these let you stay in your relationships without losing yourself.

Conclusion: Still Here

What You've Built

If you've worked through even part of this guide, you've started building something: a set of concrete skills for surviving the wait without losing the days.

You have a breathing technique for the acute moments (Chapter 1). A labeling practice to catch the rumination before it compounds (Chapter 2). A body scan to interrupt the anxiety loop before it pulls you under (Chapter 3). A sorting exercise to separate what you can control from what you can't (Chapter 4). A daily re-anchoring practice to stop pulling the rope (Chapter 5). Rituals that give the anxiety a container (Chapter 6). A time-boxing system that keeps the infrastructure running (Chapter 7). An information diet that prevents your anxiety from cherry-picking catastrophic content (Chapter 8). A minimum viable day checklist that sets a realistic bar (Chapter 9). A support calibration that matches your social needs to your social capacity (Chapter 10). Communication strategies for staying present without over-disclosing (Chapter 11). And permission to be unavailable when you need to be (Chapter 12).

None of these skills are magic. They don't make the wait pleasant, and they don't guarantee you'll feel okay. What they do is give you something to do when the anxiety arrives—which is most of the battle. You can't eliminate the discomfort, but you can stop making it worse. And that gap—the space between "not making it worse" and "collapsing"—is where you actually live through this.

The Next Step

The skills in this guide work better with practice. If you're currently in a waiting period, use the next several days as a training ground. Practice the breathing when you're not spiraling. Complete the Uncontrollable Inventory now, before you need it. Build one ritual and commit to it for seven days. Use the Minimum Viable Day checklist every evening.

The goal isn't mastery. The goal is familiarity. When the acute moment arrives—and it will—you want these skills to be things you've done before, not things you're trying to figure out in real-time.

Your next step: Download and print the **Still Here Action Checklist** (the companion PDF to this guide). It includes the full Minimum Viable Day checklist, the Uncontrollable Inventory template, the thought-labeling reference card, and a 7-day ritual tracker. Keep it somewhere visible—on your refrigerator, in a desk drawer, as a pinned note on your phone. Use it every day. The physical act of checking boxes is itself an intervention—it externalizes the process, gives you data, and creates a record of the days you made it through.

What Remains

Whatever the results turn out to be, there will be a next step. A conversation to have. A decision to make. A direction to take. This guide prepares you for the wait—not because the wait is the hard part for everyone, but because for you, right now, it is.

The days you're holding are not lost. They're just hard. And hard days are still days you live through.

Still Here.



Companion PDF: Still Here Action Checklist — includes the full Minimum Viable Day checklist, Uncontrollable Inventory template, thought-labeling reference card, and 7-day ritual tracker. Print it. Use it daily. It's designed to work alongside this guide, not replace it.